

Dashboard Metrics

Metric	What It Measures	Why It Matters	Concerning Threshold
1. Pre-Discharge Appointment Confirmation Rate	Percentage of patients discharged with a confirmed outpatient follow-up appointment documented in the care record before leaving the inpatient setting	A missing confirmation before discharge shifts the entire continuity plan from proactive to reactive, increasing coordination burden and disengagement risk for every patient who exits without a scheduled appointment	Below 95% weekly; any individual discharge with no appointment documented and no exception recorded
2. Outreach Timeliness by Risk Tier	Elapsed time between discharge and first successful case manager contact, tracked separately for elevated-risk (24–48 hr) and standard-pathway (2 working days) patients	Conflating risk tiers in a single timeliness measure masks whether the highest-risk patients — those with suicide risk, disengagement history, medication instability, or recent crisis — are receiving earlier contact as required	Any elevated-risk patient not contacted within 48 hours; any standard-pathway patient not contacted within 2 working days; risk tier undocumented at discharge
3. Structured Outreach Escalation Rate	Percentage of unreachable patients for whom structured outreach protocol was initiated, and of those, the proportion escalated to CM Lead or treating consultant within the required same-day window for high-risk cases	Without tracked escalation completion, the workflow has no mechanism to prevent silent failure when a patient cannot be reached — the most operationally dangerous gap between Decision Points 3 and 4	Any elevated-risk patient unreachable without same-day escalation documented; any patient where outreach attempts are logged but escalation decision is absent after 2 working days
4. Missed Appointment Same-Day Review Completion Rate	Percentage of missed first follow-up appointments that triggered a documented same-day case manager review and outreach initiation within the required timeframe	A missed first post-discharge appointment is the central failure event this workflow is designed to recover from; recovery depends on detection being immediate and systematic rather than reliant on individual case manager awareness	Any missed appointment without same-day review documented; outreach not initiated within 4 hours of missed appointment being recorded
5. Missed Appointment Resolution Pathway Completion	Whether the DP6 triage decision (logistics barrier vs. clinical concern) was documented, the appropriate pathway followed, and continuity restored or escalated through DP7	DP6 is a clinically significant judgment that determines whether a patient is rescheduled or referred to crisis intervention; without documented pathway completion, resolution cannot be verified and pattern failures remain invisible across the caseload	DP6 decision undocumented more than 4 hours after missed appointment confirmed; continuity unrestored at DP7 with no further escalation recorded
6. Ownership Assignment Gap at Discharge	Percentage of discharges where a named case manager with confirmed continuity ownership is documented in the care record before the patient exits the inpatient setting	Ambiguous ownership is a root cause of outreach delays and deferred escalation decisions; when no named case manager holds documented responsibility, coordination accountability collapses at the handoff point	Any discharge without named case manager ownership documented; any discrepancy between the CM listed in the care record and the CM who initiated post-discharge outreach
7. Caseload Concentration and Outreach Overdue Volume by Case Manager	Per case manager: number of active post-discharge patients within outreach windows, number of overdue outreach contacts, and number of unresolved escalation cases tracked in real time	The workflow places significant coordination burden on individual case manager vigilance; without visibility into load concentration, an unusually high volume of elevated-risk patients in a given week generates no systemic alert or supervisory response	More than 3 overdue outreach contacts per case manager simultaneously; elevated-risk caseload concentration without documented CM Lead awareness
8. Documentation Completeness for Escalation Actions	Whether each required escalation or structured outreach action — including DP6 triage decisions, structured outreach attempt records, escalation notifications, and continuity ownership confirmations — is recorded with sufficient detail to support handoff and audit	Undocumented escalation actions are operationally indistinguishable from actions that did not occur; incomplete records bypass supervisory oversight, create handoff failure risk, and prevent accurate incident review when continuity breaks down	DP6 triage undocumented within 4 hours of missed appointment; escalation initiated without confirmation record within 2 hours for high-risk cases; any case manager's active post-discharge documentation completeness falling below 90%